

TASK 3 – CAUSALITY ASSESSMENT OF DRUG REACTIONS

Structured Causality Assessment Report

Task ID: task3

1. CASE STUDY

Patient	45-year-old female
Suspected Drug	Amoxicillin
Adverse Reaction	Skin rash
Onset	3 days after starting the drug
Outcome	Recovered after stopping the drug

2. ASSESSMENT CRITERIA

Temporal Relationship	Positive. The skin rash developed 3 days after amoxicillin was initiated, providing a reasonable temporal relationship between drug exposure and the reaction.
Dechallenge	Positive. The patient recovered after amoxicillin was stopped, supporting a relationship between the drug and the reaction.
Alternative Causes	No alternative cause is provided in the case study. Therefore, an alternative explanation cannot be established from the available information.
Rechallenge	Not performed / not applicable. There is no information showing that amoxicillin was administered again and caused the rash to recur.

3. WHO-UMC CAUSALITY ASSESSMENT

Causality Category	PROBABLE / LIKELY
Reasoning	The reaction has a reasonable time relationship to drug administration, improved after withdrawal of the suspected drug, and no alternative cause is identified in the case information. Rechallenge was not available.
Why not Certain?	A 'Certain' classification generally requires stronger evidence, such as a clear and convincing clinical pattern and/or definitive rechallenge evidence where appropriate. Rechallenge is absent in this case.

4. CRITERIA SUMMARY

No.	Criterion	Finding
1	Drug exposure preceded the reaction	YES
2	Appropriate temporal relationship	YES
3	Improvement after withdrawal (dechallenge)	YES
4	Alternative cause identified	NO
5	Positive rechallenge	NOT AVAILABLE

5. FINAL ASSESSMENT

Based on the information provided, the relationship between **amoxicillin** and the reported **skin rash** is classified as **PROBABLE / LIKELY**. The strongest supporting factors are the appropriate temporal relationship and the patient's

recovery following discontinuation of the suspected drug. No alternative cause is documented, while rechallenge information is unavailable.

6. CONCLUSION

The case demonstrates a plausible causal association between amoxicillin and the skin rash. The available evidence supports a probable/likely drug-related reaction rather than a certain relationship.

Note: This is a simulated pharmacovigilance training assessment prepared from the case study shown in the task instructions. It is not a real patient medical record.